Mr. Bhavani Prasad

Age / Sex : 61 YRS / M

Referred by : INDRA KIDNEY CARE & SURGICAL CENTER

Reg. no. : 6990

6990

Registered on : 21/10/2026 11:03 AM

Collected on : 21/10/2026 10:30 AM

Received on : 21/10/2026 10:40 AM

Reported on : 21/10/2026 12:00 PM

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SEROLOGY & IMMUNOLOGY

VIRAL MARKER

TEST	VALUE	UNIT	REFERENCE
HIV (CARD TEST)			
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		
HEPATITIS C VIRUS, HCV	**POSITIVE**		
HBSAG	NEGATIVE		

Advice — *RECOMEND FOR HCV ELISA/ RNA FOR CONFIRMATION*

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

~~~ End of report ~~~

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